A Conversation About Her Life, Not About a Move

A short guide to care wishes while your wife is well.

Your wife is well. She runs her own days, and the subject of future care has never come up between you. There's no decision to make, and this guide won't ask you to make one.

It explains what a first conversation about care wishes is for: learning what she would want to keep if things ever change, and what you and the kids could honestly offer. It explains. It doesn't plan anything.

Start without forcing a decision

A stretch when nothing is happening is a good time for this. The National Institute on Aging says a conversation about what care might be wanted in the future is most productive when there is not an emergency. [1]

It begins with a question about the conversation itself: would she be willing to talk about this sometime? If she says not today, that's her answer for today.

After that, keep it simple. The National Institute on Aging's advice is to start simple, because talk of specific medical treatments or decisions can be overwhelming. AARP's planning guide for families says to just get the conversation started, to expect that it will take place over time and, most importantly, to listen. [2] [3]

Nothing she says has to be final. The Conversation Project's starter guide puts it this way: nothing you say is permanent, and you can always change your mind as things change. [4]

- Ask about everyday life: her mornings, her routines, the people and places she counts on.
- Leave moving, facilities and her health out of the first conversation.
- Treat whatever gets written down as something that can change.

Listen to her, in her words

A good first question is the one the Conversation Project asks: what does a good day look like for you? For more detail, a federal toolkit on person-centered thinking suggests asking the person to walk through a typical weekday and a typical weekend day. [4] [5]

When you write it down afterward, keep her wording. The Institute for Healthcare Improvement gives care teams the same instruction: use the person's own words as much as possible. [6]

Then read your notes for anything you concluded that she didn't say. “She'd never want strangers in the house” may well be true. Until she says it, it's your guess, and it goes on a separate line.

After many years of marriage it's natural to feel you already know her answers. You may. The Conversation Project observes that we sometimes think others know how we feel when they don't, and that conversations are what make it clear. [4]

Listen to the family

The second set of voices is yours and the kids'. Each of you has hopes and worries about the years ahead, and they may not all match. Hers stay on her page. Yours go on another.

Then ask each person what they could really offer if she ever needed more help: what, how often, and what it depends on. AARP's guide suggests each family member consider what they are prepared and not prepared to do. The National Institute on Aging says no one can be expected to do everything, and asks people to be realistic about how much they can do and what they are willing to do. [3] [1]

Circumstances move, too. The National Institute on Aging notes that family members' availability can change over time. That is one reason an honest “it depends” is worth more on the page than a promise. [1]

- Wanting to help and being able to help are two different notes. Write both.
- Where two people expect different things, write down both expectations. Don't settle them.

Keep agreement, disagreement and open questions in view

By now you have her words, the family's hopes and worries, and what each person could offer. The last part is to mark where those agree, where they differ, and what nobody knows yet.

Finding a difference isn't a setback. The Conversation Project says you may discover that you and the people you trust disagree, that this is OK, and that the important thing is to keep talking. The Family Caregiver Alliance adds that not every issue can be solved, and that the most important thing to remember about a family meeting is that it is not a one-time event. [4] [7]

Later, a family can use what it has written to agree on what any future choice would be measured against. That isn't part of a first conversation, and this guide doesn't draft it. For now the job is to keep the open questions on the page instead of letting them disappear.

What this means for your family

- This is a conversation about her life as it is now. Nobody has to agree to anything.
- Her words go down as she said them. Your conclusions go on a separate line.
- An honest “I could do Tuesdays, as long as my job stays put” is more use than “we'll take care of you.”
- It isn't finished in one sitting. Agree when you'll look at the page again.
- Questions about her health go to her doctor. Questions the family can't answer by talking go on a list for later.

What this guide doesn't do

It doesn't say whether your wife will ever need help, suggest any arrangement, or settle what the family should do. The advice attributed to an organization comes from the sources below. The four parts, and the advice about what to write down, are the Family Care Roadmap's own.

Ready for the next step?

You know what this conversation is for and why it can stay open. When you're both ready, the care interview adds the rest of the picture: it walks you through your wife's situation area by area and keeps her wishes alongside it in a Family Care Profile you can bring to any care conversation.

Create Your Free Family Care Profile ›

WHERE THIS COMES FROM

1. National Institute on Aging, Sharing Caregiving Responsibilities
2. National Institute on Aging, Advance Care Planning and Health Care Decisions: Tips for Caregivers and Families
3. AARP, Prepare to Care: A Planning Guide for Families

4. The Conversation Project (Institute for Healthcare Improvement), Your Conversation Starter Guide: How to talk about what matters to you and have a say in your health care
5. Administration for Community Living (National Center on Advancing Person-Centered Practices and Systems), Office of Healthcare Information and Counseling Person-Centered Thinking Toolkit
6. Institute for Healthcare Improvement (Age-Friendly Health Systems, with The John A. Hartford Foundation), “What Matters” to Older Adults? A Toolkit for Health Systems to Design Better Care with Older Adults
7. Family Caregiver Alliance, Holding a Family Meeting
